



SURGICAL PATHOLOGY REPORT

* Addendum *

Patient Name:

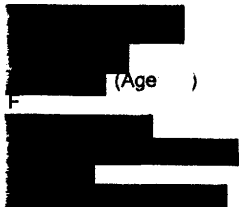
Med. Rec. #:

DOB:

Gender:

Ref. Physician:

Patient Address:



Location:

Service:

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Billing Type:

Additional Copy to:

Ref. Source:

Clinical Diagnosis & History:

1. T1/ Tis bladder cancer..

Specimens Submitted:

- 1: Bladder, bilateral tubes and ovaries and uterus, radical cystectomy, hysterectomy, and bilateral salpingo-oophorectomy
- 2: Lymph nodes, left pelvic, resection
- 3: Lymph nodes, right pelvic, resection
- 4: Round ligament, right, excision
- 5: Round ligament, left, excision
- 6: Soft tissue, perivesicle, excision
- 7: Ureter, distal left, segmental resection
- 8: Ureter, distal right, segmental resection

DIAGNOSIS:

1. Bladder, bilateral tubes and ovaries and uterus, radical cystectomy, hysterectomy, and bilateral salpingo-oophorectomy:

Tumor Type:

Invasive urothelial carcinoma, NOS

Histologic Grade:

High grade

Pattern of growth of the Non-Invasive component:

Papillary and flat

Pattern of growth of the Invasive component:

Infiltrating

Tumor Multicentricity:

Identified

Bladder Local Invasion:

Superficial half of muscularis propria

Extravesical Tumor Extension:

Right ureter uninvolved
Left ureter involved
Urethra uninvolved

Vascular Invasion:

Not identified

Perineural Invasion:

Not identified

ICD-O-3
Carcinoma, urothelial NOS
8/20/13
C67.1
Site Dome of bladder
Path
Bladder NOS C67.9
8/16/13

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Surgical Margins:

Free of tumor

Non-Neoplastic Mucosa:

Exhibiting ulceration

Exhibiting foreign body reaction

Exhibiting proliferative cystitis (Brunn's nests, cystitis cystica, cystitis glandularis)

Female Genital Organs:

The endometrium shows atrophy

The right fallopian tube shows a focal nodular epithelial proliferation (see comment)

The left ovary shows simple serous cyst

Remaining genital organs are unremarkable

Perivesical Lymph Nodes:

LN Not involved 1

The Pathologic Stage is (AJCC 2002):

pT2a (Invades superficial half of muscularis propria)

Comment: Based on the initial immunohistochemical evaluation, the approximately 2 mm nodule is consistent with a primary müllerian-derived glandular lesion, and not metastatic urothelial carcinoma. Further studies are being performed to better characterize it, and the results will be issued in an addendum report.

2. Lymph nodes, left pelvic, resection:

Lymph Nodes:

Not involved

Number of nodes examined:3

3. Lymph nodes, right pelvic, resection:

Lymph Nodes:

Not involved

Number of nodes examined:9

4. Round ligament, right, resection:

Benign segment of round ligament

5. Round ligament, left, resection:

Benign segment of round ligament

6. Soft tissue, perivesical, resection:

Benign adipose tissue.

7. Ureter, left distal, resection:

Benign segment of ureter

8. Ureter, right distal, resection:

Benign segment of ureter

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

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Special Studies:

Result

Special Stain
WT1

Comment

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P53
4A4/P63
34BE12
P53
IMM RECUT
NEG CONT
IMM RECUT
NEG CONT
MIB-1 (Ki-67)
CK5/6
D2-40
ER-C
PR-C
EMA
CALR
INHibin
GFAP
RECUT

Gross Description:

1.) The specimen is received fresh, labeled "bladder, urethra, urachus, perivesical tissue, uterus, bilateral tubes and ovaries and anterior vaginal wall". It consist of a bladder, urethra, urachus, perivesical tissue, uterus, bilateral tubes and ovaries and anterior vaginal wall measuring 10.5 cm from superior to inferior, 15.0 cm laterally and 14.0 cm from anterior to posterior. The bladder measures 9 x 8.5 x 4.2 cm, the uterus measures 5.5 x 3.2 x 1.5 cm, the right ovary measures 2.0 x 1.1 x 0.6 cm, the right fallopian tube measures 10.5 x 0.3 x 0.3 cm, the left ovary measures 2.2 x 1.2 x 1.0 cm, and the left fallopian tube measures 9.8 x 0.2 x 0.2 cm. The portion of vaginal cuff measures 7.1 x 4.0 x 0.2 cm. The distal urethral margin and the vaginal cuff margin are shaved and submitted. The ureteric stumps measure 4.0 cm and are unremarkable. The specimen is inked black and the bladder is opened anteriorly to reveal:

Mass #1 -- In the right lateral wall, is a 1.7 x 0.7 x 0.2 cm soft, ulcerated, ovoid tan-brown mass, located 2.5 cm from the left ureteric orifice and 0.2 cm from the right ureteric orifices. On sectioning, the tumor extends into but not through the muscularis propria, to a depth of 0.2 cm, 0.6 cm away from the deep margin.

Mass #2 -- In the right posterior dome, is a 1.5 x 1.1 x 0.6 cm, soft, ulcerated, irregular bordered, tan-brown mass, located 0.3 cm from the left ureteric orifice and 0.7 cm from the right ureteric orifices. On sectioning, the tumor extends into but not through the muscularis propria, to a depth of 0.6 cm, 1.2 cm away from the deep margin.

Mass #3 -- In the left posterior dome, is a 0.9 x 0.8 x 0.6 cm, rubbery, pedunculated tan-brown mass, located 2.3 cm from the left ureteric orifice and 4.0 cm from the right ureteric orifices. On sectioning, the tumor appears to abut the muscularis propria and is 2.0 cm away from the deep margin. A portion of this mass has been submitted to TPS.

Mass #4 -- In the left lateral wall, is a 0.3 x 0.2 x 0.1 cm firm, round, tan-brown, papilliferous mass, located 2.5 cm from the left ureteric orifice and 4.2 cm from the right ureteric orifices. On sectioning, the tumor appears to remain confined to mucosa and is 2.0 cm away from the deep margin.

Mass #5 -- In the lower left anteriolateral wall, abutting the trigone, is a 2.1 x 1.5 x 1.0 cm firm, centrally ulcerated, bosselated bordered sessile, tan-brown mass, located 0.4 cm from the left ureteric orifice and 2.6 cm from the right ureteric orifices. On sectioning, the tumor extends into but not through the muscularis propria, to a depth of 0.7 cm and is 1.0 cm away from the deep margin.

In addition, located in the right lateral wall, is a 2.0 x 2.0 x 1.1 cm diverticula. The remaining bladder mucosa is edematous and congested. A possible discrete perivesical lymph node is identified and entirely submitted. The uterus is bivalved to reveal a roughly triangular endometrial cavity containing a flat, tan endometrium. The myometrium is smooth, tan-white and has two, E. rubbery, round tan-white mural nodules with smooth white cut surfaces, both averaging 0.4 cm in greatest dimension. The left ovary is bisected to reveal a 1.5 x 1.1 x 0.8 cm unilocular, thin-walled, clear fluid filled cysts with a smooth tan-white inner lining. The right ovary and bilateral fallopian tubes are unremarkable. Tissue is submitted for TPS. Gross potographs are taken. Representative sections are submitted.

Summary of sections:
A UM - urethral margin

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RUM - right ureter margin
LUM -left ureter margin
M1 -- mass #1
M2 -- mass #2
M3 -- mass #3
M4 -- mass #4
M5 -- mass #5
D -- diverticula
RUO -right ureteric orifice
LUO -left ureteric orifice
LP-left posterior wall
LA -left anterior wall
RP-right posterior wall
RA -right anterior wall
GONE -trigone
DOME -dome
F -perivesical fat
U -- possible urachus
EMM -endomyometrium
CX-cervix
DVM -distal vaginal margin
RFO -right tube and ovary
LFO-left tube and ovary
LN -- possible lymph node

2.) The specimen is received fresh, labeled "Left pelvic lymph nodes" and consists of a 5.3 x 8 x 1.1 cm aggregate of soft, lobulated yellow-red adipose. Sectioning reveals three rubbery tan-pink lymph nodes ranging from 1.5 to 4.5 cm in greatest dimension. The smaller lymph nodes are bisected and entirely submitted. The largest lymph node is bisected, cut in half and entirely submitted.

Summary of sections:
BLN -- bisected lymph nodes

3.) The specimen is received fresh, labeled "Right pelvic lymph nodes" and consists of a 7.0 x 3.0 x 0.5 cm aggregate of soft, lobulated yellow-red adipose. Sectioning reveals five rubbery tan-pink lymph nodes ranging from 0.9 to 3.1 cm in greatest dimension. The two thickest lymph nodes are bisected. All lymph nodes are entirely submitted.

Summary of sections:
LN -- lymph nodes
BLN -- bisected lymph nodes

4.) The specimen is received in formalin labeled, "right round ligament", and consists of a segment of tan soft tissue measuring 0.9 x 0.4 x 0.4 cm. Entirely submitted.

Summary of sections:
U--undesignated

5.) The specimen is received in formalin labeled, "left round ligament", and consists of a segment of tan soft tissue measuring 2.5 x 1.0 x 1.0 cm. Entirely submitted.

Summary of sections:
U-- undesignated

6.) The specimen is received in formalin labeled, "perivesicle fat", and consists of multiple pieces of fibroadipose tissue measuring 3.0 x 2.0 x 0.3 cm in aggregate. Entirely submitted.

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Summary of sections:

U-- undesignated

7). The specimen is received in formalin labeled, "distal left ureter", and consists of a segment of tan tissue measuring 0.4 cm in greatest dimension. Entirely submitted.

Summary of sections:

U-- undesignated

8). The specimen is received in formalin labeled, "distal right ureter clip marks proximal margin", and consists of a ureter measuring 1.0 cm in length and 0.3 cm in diameter. The proximal margin is marked with a metal clip. The metal clip is removed and the specimen is entirely submitted.

Summary of sections:

P – proximal margin

M – middle segment

D – distal margin

Summary of Sections:

Part 1: Bladder, bilateral tubes and ovaries and uterus, radical cystectomy, hysterectomy, and bilateral salpingo-oophorectomy

Block	Sect. Site	PCs
1	CX	1
1	D	1
1	DOM	1
1	DVM	1
1	EMM	1
1	F	1
1	GONE	1
1	LA	1
1	LN	1
1	LO	1
1	LP	1
3	LT	3
1	LUM	1
1	LUO	1
2	M1	2
1	M2	1
1	M3	1
2	M4	2
2	M5	2
1	RA	1
1	RO	1
1	RP	1
3	RT	3
1	RUM	1
1	RUO	1
1	U	1
1	UM	1
2	vag	2

Part 2: Lymph nodes, left pelvic, resection

Block	Sect. Site	PCs
3	BLN	3

Part 3: Lymph nodes, right pelvic, resection

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Block	Sect. Site	PCs
2	BLN	2
3	LN	3

Part 4: SP: Round ligament, right, excision

Block	Sect. Site	PCs
1	U	1

Part 5: SP: Round ligament, left, excision

Block	Sect. Site	PCs
1	U	1

Part 6: SP: Soft tissue, perivesicle, excision

Block	Sect. Site	PCs
1	U	1

Part 7: SP: Ureter, distal left, segmental resection

Block	Sect. Site	PCs
1	U	1

Part 8: SP: Ureter, distal right, segmental resection

Block	Sect. Site	PCs
1	D	1
1	M	1
1	P	1

Procedures/Addenda:

Addendum

Date Ordered:
Date Complete:
Date Reported:

Status: Signed Out

Addendum Diagnosis

#1

On immunohistochemical staining, the incidental fallopian tube lesion expresses ER, EMA and WT-1, whereas it is negative for PR, calretinin, keratin 5/6, p53, p63, 34BE12 and D2-40. The proliferative rate with Ki-67 stain is low (<5%). Inhibin and GFAP immunostains were also performed, but the lesion was no longer present in the slides stained. These results support fallopian tube epithelial differentiation. The fallopian tubes were entirely submitted for histologic examination and additional abnormalities are not identified.

The lesion is, therefore, best considered a focus of epithelial hyperplasia.

This part of the specimen was reviewed in consultation with Dr. [redacted] who concurs. Representative slides were also reviewed at the daily intradepartmental pathology consensus conference.

*** Report Electronically Signed Out ***

Signed out by [redacted]

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History	☒	☒
Dual/Synchronous Primary	☒	☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials:	STH	Date Reviewed: 5/9/13